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SciScore Report

Below you will find your SciScore report containing three tables. Your score is calculated based on adherence to scientific rigor criteria (Table 1) and identification of key biological resources (Table 2). Table 3 contains statistical tests and oligonucleotides but is not scored. If SciScore makes any mistakes, please contact us to help us learn and improve.

Table 1: Rigor Adherence Table

Ethics
Consent: All participants provided written informed consent.
IRB: The study protocol was approved by the Institutional Review Board of Taipei Veterans General Hospital (201307011A).
Inclusion and Exclusion Criteria
Participants were excluded if they had (1) a contraindication for rTMS treatment: presence of cardiac pacemakers or implantable cardioverter defibrillators, metal or magnetic objects in the brain, ear implants, aneurysm clips or coils, metal plates in the headbrain, and medication pumps for people (2) a history of epilepsy (3) visual defect or achromatopsia or (4) the presence of other neurological diseases such as Parkinsons disease or dementia.
Attrition
not detected.
Sex as a biological variable
not detected.
Subject Demographics
Age: To be included in the study, participants with stroke had to satisfy the following criteria: (1) experienced a single stroke, with 2 months to 2 years since onset (2) age between 30 and 80 years and (3) hemiparesis with upper limb weakness and functional impairment.
Randomization
The randomization order was computer-generated and concealed in sequentially numbered opaque envelopes by an independent statistician.
Blinding
Clinical assessment was conducted by an experienced physician blinded to patients group allocation, and rTMS was performed by another investigator who was not involved in clinical assessment, patient follow-up, or data analysis. Fig. 1 CONSORT flowchart fNIRS Imaging System A multichannel wearable fNIRS imaging system (NIRSport, NIRx Medical Technologies, LLC, Glen Head, NY, USA) was used to simultaneously acquire dual-wavelength (760 and 850 nm) signals.

Power Analysis
not detected.
Replication
not required.

Table 2: Key Resources Table

Your Sentences	REAGENT or RESOURCE	SOURCE	IDENTIFIER
Software and Algorithms			
The G*Power program (v3.1.9.2 Franz Faul, University of Kiel, Kiel, Germany) was used to calculate the necessary sample size 22.	G*Power		Suggestion: G*Power, RRID:SCR_013726(link)
The fNIRS signal preprocessing , including the motion artifact correction , bandpass filtering , and conversion of HbO and HbR , were processed using the HOMER2 package 30 .	HOMER2		Suggestion: Homer2 (RRID:SCR_009586)(link)
The calculation of signal CVs and correlation analyses were performed using homemade scripts developed on MATLAB (MathWorks , Natick , MA , USA) .	MATLAB		Suggestion: MATLAB, RRID:SCR_001622(link)
Statistical Analysis Statistical analysis was performed using SPSS 22.0 software (IBM Corp., Armonk, NY, USA).	SPSS		Suggestion: SPSS, RRID:SCR_002865(link)

Other Entities Detected

Your Sentences	Recognized Entity
Statistical Tests	
In the one-way analysis of variance (ANOVA) test, an a-error of 0.05 and a b of 0.20 (power level of 0.80) were used, indicating a minimum of 39 patients.	analysis of variance
	ANOVA
Demographic data including sex, involved laterality, type of stroke, and location of lesion were compared by group using a chi-square test.	chi-square test
For intragroup comparison, we used a paired t test to evaluate the group-wise motor improvement.	t test
To compare the change scores of motor performance among the three groups, we used one-way ANOVA and a post hoc analysis with Bonferronis correction.	Bonferronis
To clarify the relationship between the distance of the fNIRS-HS from the mean MEP-HS and the motor improvement, we correlated the distance and change scores of motor performance in the fNIRS group using Spearmans correlation analyses.	Spearmans correlation

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For a full description of scored criteria and tips for improving your score, please see https://www.scicrunch.com/sciscorereport-faq